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NEUROPSYCHOLOGICAL EVALUATION / PARENTAL ASSESSMENT

NAME: Ashle A. Penn
DOB: 02/15/1988

DATE OF EVALUATION: 10/30/2021

REFERRAL: Ashle Penn is a 32-year-old woman who is the mother of three children and whose youngest child, born in 2019, is in the custody of Child Welfare and placed in relative foster care. According to the caseworker, Ms. Penn is seen by social service providers as intelligent and confident and has demonstrated very good parenting skills with her children. They also describe her as becoming defensive when asked about anything to do with her Child Welfare case and describe her as being difficult to have productive conversations with due to this. The DHS caseworker's letter states that they are often surprised by Ms. Penn's decisions, and they reportedly do not appear to line up with what she expresses. They describe her as not seeming to have an accurate perception of the reality of her situation. The purpose of the current evaluation is to aid in understanding the reason behind her decision making, and she has been previously court-ordered to participate in an evaluation since December of 2020, missing two previous appointments. Information about Ms. Penn's mental health is unknown, and it is unclear to Child Welfare whether she has had any previous mental health diagnoses. The caseworker's letter goes on to report that they have little understanding about Ms. Penn's family background. Ms. Penn's father moved to Portland, Oregon in 2021 to help her regain custody of her son. After two visits, her father reportedly abruptly left and told Child Welfare he could no longer participate and support Ashle due to concerns he had for her but did not elaborate about what he meant. Child Welfare reports that Ms. Penn has previously been in a relationship with the father of her two oldest children which was marred by domestic violence, although, as will be seen below, she denies this. She is described as having been more cooperative with her ex-partner in recent years. A primary concern on the part of Child Welfare, according to the caseworker's letter, is that according to them Ms. Penn seems to have a limited ability to understand what the children have witnessed and how this may have affected them psychologically. According to the caseworker's letter, Ms. Penn has not acknowledged the violence that has been inflicted on her in front of her children. There are a number of specific questions posed by the referring caseworker which I will attempt to address at the conclusion of the narrative below.

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PROCEDURES: Ms. Penn was evaluated using clinical interview, review of available records, and the measures listed below.

1. Wechsler Adult Intelligence Scale-IV.
2. Wide Range Assessment of Memory and Learning-2.
3. Grooved Pegboard Test.
4. Boston Naming Test.
5. Delis-Kaplan Executive Function System – Trail Making Test, Conditions 1-5.
6. Delis-Kaplan Executive Function System – Verbal Fluency Test.
7. Delis-Kaplan Executive Function System – Color Word Interference Test.
8. Wisconsin Card Sorting Test – Computer Version 4.
9. Child Abuse Potential Inventory.
10. Millon Clinical Multiaxial Inventory-IV.

In addition to the above-mentioned psychometric instruments, the following records were reviewed.

1. Family Report from Multnomah County – 09/03/2021
2. DHS Assessment Summary – 11/14/2019
3. Washington County Sheriff's Office Reports

INFORMED CONSENT: Ms. Penn was informed that the contents of our diagnostic interview and psychometric assessment would be shared in a detailed report with Child Welfare and potentially the courts. She agreed to proceed with the exam.

BACKGROUND / SOCIAL HISTORY: Ms. Penn reports that she was born and raised in Southern California, moving to Portland, Oregon at age 17, where she completed her final year of high school. Her father, Avery Penn, now age 56, was a naval officer working in electronics during her childhood years, and her mother, Marie Penn, worked in medical office administration. She has a 35-year-old sister, Brittney, and a younger brother, Avery Penn Jr., age 29. Ms. Penn also has a half-brother, Masai Penn, age 8. Her parents separated when Ms. Penn was 26 years of age. Ms. Penn describes a normal childhood absent of any abuse or neglect on the part of her parents or any other family members.

Ms. Penn reports graduating from Roosevelt High School here in Portland with an A average and attending Portland Community College and Portland State University, obtaining a bachelor's degree in business and communication. She says at age 18 she worked for her father for a year as a loan originator and then for several years as a call

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center employee for Wells Fargo. During her college years she worked as a waitress and bartender for three years. This was followed by three years of human resources work as a staffing manager and recruiter, at which she is now currently employed. She states that she is working 8-5, Monday-Friday, and sometimes on weekends.

Ms. Penn reports an arrest for DUII in 2012 and participation in what sounds like a diversion program for six weeks of alcohol and drug treatment. This is somewhat at odds with the legal history given in DHS records, which says that Ms. Penn has had two DUIIs and an assault charge resulting from one of them. She is notably listed as the victim on multiple police reports relating to her former domestic partner Barry Washington.

With regard to relationship history, Ms. Penn reports that at age 18 she commenced a relationship with an Alquon Davis, then age 26, and they were a couple for a decade, separating when she was 28 years of age. She says they are still friendly with one another. Their union produced two daughters: Amaiya, age 12, and Almira, age 10. Those two children are currently placed with Mr. Davis during the week, and he has legal custody. She denies any domestic violence occurring in this partnership. Prior to the onset of the Child Welfare case, Mr. Davis did not have legal custody of the children. Ms. Penn reports that at age 28 she commenced a relationship with a Barry Washington, and they were together for four years, separating approximately two years ago. She says that there were repeated episodes of domestic violence occurring over the 12 months prior to their separation. Their union produced a son, Feliciano, who was born in January of 2019. Ms. Penn reports that Mr. Washington was arrested for domestic violence on multiple occasions and is currently on probation related to a conviction for assault of Ms. Penn. They have no contact with one another. Ms. Penn states that she is currently living alone about five minutes by car from her daughters. Feliciano is placed with a paternal aunt in Vancouver, Washington and she has been having visitation with Feliciano once a week for an hour at the DHS office but now an hour a week in the community which is supervised.

PERTINENT MEDICAL HISTORY: Ms. Penn reports having all three of her children by cesarean section. She denies any history of serious injuries or illnesses. She denies any medication allergies. She takes no current medications.

SUBSTANCE USE HISTORY: Ms. Penn reports on interview that her first use of alcohol began at age 21 and she denies any history of alcohol dependence. She describes her typical pattern of use as consuming a bottle of wine over a period of two weeks, usually drinking socially with others. She reports using cannabis on an intermittent basis between ages 18 and 27 and says her last use was two years ago. She denies use of any other illicit

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substances and denies participation in any substance use treatment other than the diversion program which followed her DUII arrest in 2012. According to the caseworker's letter, DHS has received information from collaterals about potential substance use by Ms. Penn, but this has not been confirmed and there was a report apparently in 2019 of an attempted overdose, which Ms. Penn denies on interview with me.

MENTAL HEALTH HISTORY: Ms. Penn denies any history of psychiatric hospitalizations, suicide attempts, or self-harm behaviors. She denies any history of eating disorder. She does report experiencing post-partum depression after the birth of her first child, which was successfully managed with the help of a parent mentor and a community health nurse who aided her in the early stages of her oldest child's development. She says that she has had 17 sessions of one-on-one domestic violence counseling through a therapist whom she states told her that she thought no further individual counseling was required. The history given by Ms. Penn is that she was referred to Lifeworks and no treatment was recommended, although the court has still mandated that she obtain more counseling. She says that in 2018, prior to the DHS case, she had gone to seek counseling at Lifeworks to discuss strengthening her parenting abilities and attended a year's worth of a parenting class. She states that when the children were removed in 2020 she went back to Lifeworks, and she was told that she did not meet their criteria for participation in their program, but she says Child Welfare did not believe her. Ms. Penn states that she obtained a letter from Lifeworks asserting her positive participation. After receiving another mental health referral from Child Welfare and attending an intake, she reports being told by providers that it was not medically necessary. She has been told, by her report, that the 17 sessions she attended were insufficient and has been referred again for more individual counseling but has not yet obtained a connection to do this.

I was able to speak by telephone with Sherri Lynn Blanton, QMHP, who is a counselor working for a licensed clinical social worker. Ms. Blanton states that she saw Ms. Penn this year when she had been referred for domestic violence counseling. Ms. Blanton felt that Ms. Penn met criteria for a diagnosis of adjustment disorder with anxiety. She states that Ms. Penn had denied seeing the father of her youngest child, but appeared not to be clear what the requirements were from Child Welfare. She states that most of the sessions were spent allowing the patient to vent her complaints about Child Welfare. She states that she gave her resources and eventually told her there was nothing else she thought they could do. She reports they had 15 visits altogether. They ended with the patient saying she was not going to be talking with her ex-partner Barry anymore and that they were not in contact. Ms. Blanton says her impression was that Ms. Penn was not entirely forthcoming about the nature of her situation.

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CURRENT COMPLAINTS AND FUNCTIONING: When asked about mental health symptoms, Ms. Penn states that her primary concern is that she misses her children. She denies any significant mental health symptoms, rating her mood as 8 on a scale from 0-10 most of the time, and she denies any suicide ideation. She denies any psychotic symptoms such as auditory or visual hallucinations. She denies any compulsive rituals or obsessive thoughts. She denies any trauma history such as sexual or physical assault, aside from that occurring in the context of her domestic partnership in which she was the victim of domestic violence. She denies any history of panic attacks and denies any anxiety other than that associated with her Child Welfare case. She denies any symptoms of post-traumatic stress disorder. She denies problems with sleep, saying she works late but wakes at 6am and feels rested on waking. Her appetite is normal, and she reports eating two meals a day.

BEHAVIORAL OBSERVATIONS / MSE: Ms. Penn is a right-handed, 32-year-old, African American woman, who is 5'4" tall and reports her weight at 135 lbs. She is dressed in casual clothing appropriate for the season and shows good self-care, grooming, and personal hygiene. Her speech is normal rate and productivity, with no abnormal prosody or dysarthria. She shows a normal range of affect. There were no gross motor abnormalities. Ms. Penn was cooperative to both diagnostic interview and psychometric testing and appeared to make her best effort on test tasks. Results obtained from neurocognitive assessment can be presumed to reflect an accurate picture of her abilities.

TEST FINDINGS

OVERALL INTELLECT: Ms. Penn was administered the Wechsler Adult Intelligence Scale-IV (WAIS-IV). On this measure she obtained a full scale IQ score in high average range, at the 86th percentile. High average range scores were obtained on WAIS-IV indices of verbal comprehension and working memory. She had an average range score on the WAIS-IV index of perceptual reasoning ability and her processing speed index score fell in very superior range. The reader is referred to the table below for a summary of WAIS-IV composite and subtest scaled scores, which will be discussed in greater detail as they apply to specific domains of cognitive functioning outlined below.

WAIS-IV Composite	Composite Score	Percentile Rank
Verbal Comprehension	110	75
Perceptual Reasoning	98	45
Working Memory	111	77
Processing Speed	140	99.6
Full Scale	116	86

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WAIS-IV Subtests	Scaled Score	Percentile Rank	WAIS-IV Subtests	Scaled Score	Percentile Rank
Similarities	11	63	Visual Puzzles	9	37
Vocabulary	13	84	Digit Span	10	50
Information	12	75	Arithmetic	14	91
Block Design	9	37	Symbol Search	16	98
Matrix Reasoning	11	63	Coding	19	99.9

MOTOR FUNCTION: Ms. Penn showed no deficit in fine motor control or motor speed. On the Grooved Pegboard Test, she obtained a high average range score of 63 seconds with her right (dominant) hand and 59 seconds with her left hand. On the Delis-Kaplan Executive Function System (D-KEFS) Trail Making Test Condition 5, a measure of motor speed, her score was also high average (SS=13).

LANGUAGE BASED FUNCTION: Ms. Penn shows a high average range vocabulary, consistent with her measured overall intellect. This was replicated in her performance on the Boston Naming Test in which she was asked to verbally identify a series of 60 objects presented in pictures. She obtained an average range score, with 54 of 60 spontaneously given correct responses and 59 of 60 correct responses with phonemic cueing. Ms. Penn also showed no deficit in oral fluency and verbal retrieval, in fact scoring in above average range on the D-KEFS Verbal Fluency Test and gave a superior range score when cognitive load was increased (D-KEFS Verbal Fluency: Condition 1 SS=15, Condition 2 SS=14, Condition 3 SS=19).

ATTENTION / CONCENTRATION: Ms. Penn showed no deficit in attention span, with an average range performance on WAIS-IV digit span. She was able to repeat 8 digits forward and had a raw score of 6 on sequencing and also fell in average range on digits backward. This was replicated in her performance on both the letter number repetition and sentence memory subtests of the Wide Range Assessment of Memory and Learning-2. Ms. Penn was also administered the D-KEFS Trail Making Test Conditions 1-4. On this measure she showed superior range function on a visual scanning task and scored in high average range on both number and letter sequencing tasks. Her score on Condition 4 of the Trail Making Test, which requires both divided attention and sequential reasoning, fell in average range with 1 error of set loss (D-KEFS Trail Making: Condition 1 SS=15, Condition 2 SS=13, Condition 3 SS=13, Condition 4 SS=11).

VISUALLY BASED SKILLS: As noted above, Ms. Penn showed no deficit on a visual scanning task. She performed in average range on tests requiring constructional ability and mental modeling of the assembly of parts into wholes. On the matrix reasoning subtest of

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the WAIS-IV, a visually based analytic and fluid reasoning task which requires novel problem solving using visual material, her score was in the middle of average range.

REASONING AND JUDGMENT: Ms. Penn showed a solidly average range capacity for abstract reasoning, consistent with her measured overall intellect and background and education. Her fund of information was high average compared to same-age cohorts. There was no indication, based on diagnostic interview, that she has any difficulty recognizing ordinary social mores.

LEARNING AND MEMORY: Ms. Penn was administered the Wide Range Assessment of Memory and Learning-2 (WRAML-2). This is a broad range battery of both verbal and visual memory tests which includes both immediate and delayed recall trials, as well as tests of recognition memory for both verbal and visual material. Ms. Penn showed no deficit on this measure in either verbal or visual memory on either immediate or delayed recall trials, or on tests of recognition memory. The reader is referred to the table below for a summary of WRAML-2 core subtest scaled scores (scaled score of 7-13 is average).

<u>WRAML-2 Core Subtest</u>	<u>Scaled Score</u>
Story Memory	15
Design Memory	15
Verbal Learning	8
Picture Memory	9
Number Letter	14
Sentence Memory	10
Story Memory Recall	14
Verbal Learning Recall	9
Story Recognition	12
Design Recognition	15
Picture Memory Recog.	9
Verbal Learning Recog.	9

EXECUTIVE FUNCTIONING: As noted above, Ms. Penn scored within normal limits on a task requiring both divided attention and sequential reasoning. She showed superior range processing speed on both symbol search and coding subtests of the WAIS-IV and had a superior range score on the arithmetic subtest of the WAIS-IV showing again, no difficulties with working memory. She again showed normal range processing speed on the D-KEFS Color/Word Interference Test on both color naming and word reading tasks. On Condition 3 of the Color/Word Interference Test, which requires response inhibition, her score was high average, and when cognitive load was increased and she was required to perform a task which involved both response inhibition and category switching in

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combination, her score was again at the top end of average range (D-KEFS C/W Interference Test: Condition 1 SS=12, Condition 2 SS=14, Condition 3 SS=13, Condition 4 SS=13).

Ms. Penn was additionally administered the Wisconsin Card Sorting Test – Computer Version 4 (WCST). In this test the subject is asked to match a series of cards appearing on a computer screen by using different strategies such as color, shape, or number. The demand for matching strategy is shifted unexpectedly over the course of the test's administration and the test is thought to measure the subject's capacity for conceptual reasoning, mental flexibility, and shifting of cognitive set. Ms. Penn scored in average range on this measure in terms of her ability to avoid making errors and in her ability to avoid making errors when categories were switched unexpectedly. She recognized 6 of 6 categories (WCST: TE standard score = 102, PE standard score = 98, Categories = 6 – using age and education demographically corrected norms).

PERSONALITY FUNCTIONING ASSESSMENT: Ms. Penn was administered the Millon Clinical Multiaxial Inventory-IV (MCMI-IV). No specific clinical syndromes such as mood, anxiety, or thought disorder emerge from the profile. There are elements of histrionic and narcissistic personality, but Ms. Penn was mildly defensive in her responding. Individuals with this profile tend to display at times impetuous behavior and yet have a high self-regard, viewing themselves as highly reliable and as disciplined in their communication. This profile is obtained by two groups of individuals. The first group includes well-functioning, generally adaptive individuals with no major personality disturbances who may be experiencing current psychosocial stressors. The second group of people show some behaviors which others may find difficult to tolerate. Typically, individuals with this profile have little in the way of uncertainty and while adept at winning over the favor of others initially, many relationships may not be long lasting. Individuals with this profile are often also unwilling to examine their own role in difficult situations that they find themselves in which involve prolonged distress and may respond by behaving in a somewhat erratic fashion. She is more novelty-seeking than average. Most notable is that this profile is someone with a very forceful and driven energy level who is likely to be quite ambitious, leading a highly structured and strictly organized existence. Individuals with this profile are unlikely to seek individual psychotherapy.

DHS CASE / PARENTING: I asked Ms. Penn to convey the narrative of how she became involved with Child Welfare and the nature of her child's placement in substitute care. She states that she did have an involvement with a Child Welfare caseworker in 2019, but after an initial contact, this individual never came to see her. She states that in January of 2020

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a caseworker named Carissa came to the house and was accompanied by police. The caseworker assumed that she was in danger from her partner Barry, who had been arrested four days prior. Ms. Penn says she told the police she was okay when they arrived at first, but then the caseworker Cassie arrived. She reports that at the time she had a domestic violence grant and she had suitcases packed, intending to go to Las Vegas along with her son to where family was living. The worker questioned why Barry was in the house and Ms. Penn says that it happened to be Feliciano's first birthday. The caseworker then obtained a warrant and removed the children after the patient refused to allow her to inspect the home. Feliciano was at his paternal grandmother's house for the weekend at the time. The children were removed on February 29th. Ms. Penn's understanding is that the allegation was that she was failing to protect the children from exposure to domestic violence and she learned at a hearing in February that she was alleged to have refused services, something she vehemently denies. When I asked why her one-year-old has still not been returned after nearly two years, she says her belief is that Child Welfare fears she will allow contact with the father. The two older girls were removed on January 29, after which Ms. Penn placed them with their father to avoid their becoming involved in the social services system. She states that Child Welfare has mandated that she seek mental health services but was told by Lifeworks at an intake last year, by her report, that there is no need for services. I asked her to discuss the issue of her father coming to visit her and subsequently leaving rather abruptly. She says that she got into an argument with her father over her telling her 8-year-old half-brother, her father's son, not to spit on one of the girls when they were in the backseat of the car. Her father reacted to her angrily saying that she was not to discipline his child. They were not able to resolve their conflict and her father left and returned to the eastern seaboard. We went on to discuss her relationship with Child Welfare and she describes this as conflicted. She disagrees with the statement by the caseworker that she had said her son was not affected by exposure to domestic violence. She says that this is not what she intended to convey. She states that she has now had five hearings and the plan has been return-to-parent up until the last hearing. She has a new attorney and says she has discharged several past attorneys, feeling that they were not adequately representing her. There is another hearing scheduled for January. At this point, she says she has had 17 visits of individual therapy with a therapist named Sherri Lynn Blanton. She says that she has also been referred to Wolfpack as a mandated service through Child Welfare and was supposed to see a counselor named Klarissa Gaskell but ended up not seeing her. There has been no referral to a parenting class, and she says DHS has never felt that she had problems with her parenting or parent child interaction. She was later referred, after the Wolfpack referral, to the Domestic Violence Resource Center where she did an intake but was told that they could not offer what Child Welfare was asking for, which was more than the 10 sessions of supportive counseling that the domestic violence

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resource center offers. While there, she talked with a Saba Attar. I asked her to talk about the outcome of her therapy with Sherri Lynn Blanton and she said that the primary focus of counseling was on psychological self-care, prioritizing things in her life, and recognizing her own agency in making decisions about her life. She does not recall any mental health diagnoses being made.

We went on to talk about her relationship with her youngest son's father. She says they have no contact at present and he is on probation. She is aware that he has visitation similar to her own, which is once a week at the Child Welfare office and which is supervised. She does not think they would ever reconcile but believes that they could cooperatively coparent as separated parents. We went on to talk further about the effect of all of these events and the domestic violence that the girls witnessed has had on them. She says that they witnessed only one incident of domestic violence and states that the girls were not physically abused by her partner Barry. She states that she does not believe the girls are showing symptoms of maladjustment or post-trauma syndrome. She is concerned that the girls' father, Alquon, with whom the girls are staying, is physically abusive to the girls and that he is not attending to their hygiene and often leaves them alone.

With regard to parenting practices, Ms. Penn answered questions regarding ordinary child development expectations and parenting practices in a coherent and constructive way. I also asked her to complete the Child Abuse Potential Inventory (CAPI). This is a broad range self-report measure which yields a subscale indicating the overall general risk of the possibility of abusive behavior in the parenting role, as well as containing subscales suggesting the presence of distress, rigidity in her coping style, unhappiness, problems in relationship with the child, problems with family, and an ego strength subscale. I was not able to obtain a valid profile on this measure and the cutoff score for the so-called "lie" scale was exceeded. Perusal of specific items however did not suggest to me that Ms. Penn was significantly at risk for harming her children. She was consistent in her responding and there was no evidence of random responding. The rigidity subscale of the CAPI was mildly elevated, consistent with what appears to be some elements of compulsive personality, and the ego strength subscale was significantly elevated, suggesting that Ms. Penn may at times have a rather inflated view of herself.

SUMMARY / FORMULATION: This 32-year-old woman is the mother of three children, one of whom, a 3-year-old boy, has been placed in the custody of Child Welfare for nearly the last two years. The children were removed in early 2020 as a result of concerns about their exposure to domestic violence from Ms. Penn's former domestic partner, who is the father of her youngest child. Child Welfare has continued to have concerns about Ms.

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Penn's ongoing contact with this man, although she claims they do not have any. This is at odds with information obtained from Child Welfare which reports that there have continued to be episodes in which they are involved with one another, potentially representing a danger to her youngest child. She currently has visitation once a week in the community but previously supervised had visitation over most of the last two years for one hour a week at the DHS office. Her 3-year-old son is placed with his paternal grandmother. Ms. Penn arranged for her oldest two children to be given to the custody of their father at the beginning of the case and they are not a part of the current Child Welfare involvement. Ms. Penn describes a childhood in which she was mostly raised in Southern California but finished high school in Oregon and has obtained a bachelor's degree in business and communications through Portland State University. Her parents separated when she was 26 years of age, and she has an 8-year-old half-brother. She denies any history of being a victim of abuse or neglect but describes her father as a military man with a compulsive style. Ms. Penn reports an arrest for driving under the influence of intoxicants in 2012 and participation in a diversion program. Ms. Penn had an assault charge associated with one of these DUII charges and she is listed as the victim on multiple police reports relating to violence from her former domestic partner Barry Washington. Ms. Penn also describes a somewhat conflicted relationship with her ex-partner Alquon Davis, the father of her two daughters, aged 12 and 10, insofar as she sees him as a poor parent who has at times been physically abusive toward the children and not attended to their personal hygiene and care.

Medical history is unremarkable. Ms. Penn describes first using alcohol at age 21 and denies any history of alcohol dependence. She has a history of intermittent cannabis use by her report between ages 18-27. She denies use of any other illicit substances. Ms. Penn denies reports given to Child Welfare that she made a suicide gesture by attempted overdose in 2019. Her mental health history is reportedly absent of any psychiatric hospitalizations, suicide attempts, or self-harm behavior. She does report an episode of post-partum depression after the birth of her first child, which she says was significantly mitigated by involvement with a parent mentor and Child Development specialist. She has had referrals for mental health counseling and reports attending a 17-session course of therapy, who gives a mixed report of her involvement in treatment and participation in counseling. She describes having other referrals to Lifeworks and to the Domestic Violence Resource Center which were not completed as a result, by her report, of inconsistency between the offerings of these programs and the mandate from Child Welfare and the courts.

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Ms. Penn denies any current mental health symptoms, although she reports distress about missing her children. She denies any trauma history and describes normal sleep and appetite. Ms. Penn presented with a normal rate and rhythm of speech and no abnormal motor behavior. She shows good self-care, grooming, and personal hygiene, and a normal range of affect. She displayed some carelessness in her approach to the evaluation at times, taking a longer lunch than was anticipated and arrived somewhat late as a result of traffic. Ms. Penn's measured overall intellect falls in high average range. She shows no deficit in fine motor control or motor speed and high average to superior range functioning in the domain of language. There is no evidence of any deficit in attention span, and she shows no impairment in sequential reasoning. Visual spatial ability also falls well within normal limits. She shows a solidly average range capacity for abstract reasoning and a high average range fund of information. Tests of learning and memory for both visual and verbal material show no deficit. Tests of executive functioning were similar, insofar as there is no evidence of any deficit in her capacity for processing speed, mental flexibility, set shifting, or planning and organization. A measure of personality functioning was answered in a mildly defensive fashion but did not indicate the presence of any clinical diagnoses. She has a personality style characterized by compulsive and paranoid personality traits and is highly mistrustful of others. She appears to display a degree of inflated self-regard. Individuals with her profile are sometimes unwilling to examine their own role in complicated or adverse circumstances in which they find themselves. The narrative of her involvement with Child Welfare was somewhat hard to follow but she has now dismissed several attorneys representing her and has had a fraught relationship with a variety of caseworkers and social service providers. She describes her relationship with her youngest child's father as estranged, at odds with reports from Child Welfare, and claims no intent to reconcile with him but believes they could coparent responsibly as separated individuals. A self-report measure related to parenting and the overall risk of abusive behavior did not yield a valid profile, but specific perusal of item endorsement did not indicate that Ms. Penn is at risk of harming her child.

With regard to the specific questions posed by the referring caseworker, I find no evidence of any cognitive deficits or intellectual difficulties. In fact, Ms. Penn scores in high average range overall. There are no clinical diagnoses evident but features of compulsive and paranoid personality. Ms. Penn's insight into the conditions which led to her children being in custody appears to be fair. Ms. Penn's perception of her own strengths and weaknesses is that she sees herself as a highly organized and functional individual who has had inadequate representation and/or treatment by social service providers. I have no way of establishing the veracity of her self-report with regard to her contact with her ex-partner, which is quite at odds with that given by Child Welfare. A referral for mental health

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counseling would not be in the service of treatment of mood, anxiety, or thought disorder, nor is there evidence for a severe disorder of personality. I do suspect that the personality traits evidenced by Ms. Penn do contribute to some conflicted interactions with legal representation and social service providers. I do not see any evidence in the profile or in diagnostic interview that Ms. Penn has any difficulty recognizing ordinary stages of child development or that she has any unusual attitudes toward parenting practices and discipline.

DSM-5 DIAGNOSES

1. Features of compulsive and paranoid personality.

RECOMMENDATIONS:

1. Mental Health Services: At this point, I do not recommend participation in substance abuse treatment or mental health counseling, given the lack of evidence for mental disorder. In spite of her history of being a victim of domestic violence, I do not see Ms. Penn as suffering from symptoms of post-traumatic stress disorder.
2. Parenting: Ms. Penn, in my opinion, does not merit referral for parent training. She seems to have an adequate understanding of child development and parenting practices which would likely not be improved by attending the typical parenting class. I do not see strong evidence that she would be unable to put the needs of her children ahead of her own. Nevertheless, Ms. Penn's mixed ability to understand the nature of Child Welfare and the degree to which she has been frank and transparent with social service providers appears to be a complicating factor in the case. I am not in a position to ascertain the veracity of Ms. Penn's statements to social services providers; however, overall, her mental health evaluation does not suggest significant psychiatric illness of any kind.
3. Neuropsychological Follow-Up: It is recommended that Ms. Penn have a follow-up session with this provider in order to review in detail the findings of the current psychological examination.

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Thank you for the opportunity to evaluate this interesting woman. Please let me know if I can be of further help in this matter.

A handwritten signature in black ink, appearing to read 'C. Tongue', with a stylized, flowing script.

Christopher K. Tongue, Ph.D.

CKT/jt